CARDIOVASCULAR RISK ASSESSMENT

Collected on Jul 15, 2016 8:26 AM

If you have questions or concerns regarding your test results, contact the clinician who ordered the test.

Results

Performed at: 02 - Litholink Corporation
2250 West Campbell Park Drive, Chicago, IL 606123502
Lab Director: Mitchell Laks PhD, Phone: 3122430600

Interpretation

Value
Note

CARDIOVASCULAR REPORT:

Current available clinical information suggests the patient's risk is at least LOW. If the patient has two or more major risk factors, the risk category is intermediate. If the patient has CHD or a CHD risk equivalent, the risk category is high. If patient does not have CHD or a CHD risk equivalent, consider use of the Pooled Cohort Equations to estimate 10-year CVD risk, as individuals with greater than 7.5% risk may warrant more intensive therapy. The calculator can be found at:
http://tools.cardiosource.org/ASCVD-Risk-Estimator/
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Insulin resistance, obesity, excessive alcohol use, smoking, thyroid disease, nephrotic syndrome, liver disease, and certain medications are all causes of secondary dyslipidemia. Consider evaluation if clinically indicated.
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Therapeutic lifestyle changes are always valuable to achieve optimal blood lipid status (diet, exercise, weight management).

LIPID MANAGEMENT
Select one patient risk category based upon medical history and clinical judgment. Additional risk factors such as personal or family history of premature CHD, smoking, and hypertension modify a patient's goals of therapy. In CVD prevention, the intensity of therapy should be adjusted to the level of patient risk. MODERATE intensity statin therapy generally results in an average LDL-C reduction of 30% to less than 50% from the untreated baseline. Examples include (daily doses): atorvastatin 10-20 mg, rosuvastatin 5-10 mg,

simvastatin 20-40 mg, pravastatin 40-80 mg, lovastatin 40 mg. HIGH intensity statin therapy generally results in an average LDL-C reduction of 50% or more from the untreated baseline. Examples include (daily doses): atorvastatin 40-80 mg and rosuvastatin 20 mg.

LOW RISK ASSESSMENT AND TREATMENT SUGGESTIONS

LDL-C is acceptable, 144 mg/dL.
Non-HDL Cholesterol is
acceptable, 159 mg/dL.

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Considerations for use of statin therapy include family history of premature atherosclerotic disease, elevated coronary artery calcium score, ankle-brachial index < 0.9, elevated CRP, or elevated 10-year or lifetime CVD risk.

INTERMEDIATE RISK ASSESSMENT AND TREATMENT SUGGESTIONS

LDL-C is borderline high, 144 mg/dL. Non-HDL Cholesterol is acceptable, 159 mg/dL.

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Consider beginning or increasing statin. Factors that may influence statin use include family history of premature atherosclerotic disease, elevated coronary artery calcium score, ankle-brachial index < 0.9, elevated CRP, or elevated 10-year or lifetime CVD risk. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.

HIGH RISK ASSESSMENT AND TREATMENT SUGGESTIONS

LDL-C is high, 144 mg/dL. Non-HDL Cholesterol is borderline high, 159 mg/dL.

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Begin statin. If statin already in use, consider increasing dose to achieve at least a 50% LDL reduction from baseline. Moderate or high intensity statin is preferred. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.

DISCLAIMER

These assessments and treatment suggestions are provided as a convenience in support of the physician-patient relationship and are not intended to replace the physician's clinical judgment. They are derived from the national guidelines in addition to other evidence and expert opinion. The clinician should consider this information within the context of clinical opinion and the individual patient.

SEE GUIDANCE FOR CARDIOVASCULAR REPORT: National Heart, Lung, and Blood Institute's Third Report of the NCEP Expert Panel on Detection
, Evaluation and Treatment of High Blood Cholesterol in Adults (ATP III) (2002. NIH publication 02-5215); Brunzell et al. Diabetes Care 2008; 31(4):811-82; Contois et al. Clin Chem 2009; 55(3):407-419; Stone N J et

Controls et al. Clin Chem. 2009;55(3):407-419; Stone NJ et al. 2013 ACC/AHA guideline on the treatment of blood cholesterol to reduce atherosclerotic cardiovascular risk in adults: a report of the American College of Cardiology/American Heart Association Task Force on Practice Guidelines. Circulation 2014;129(suppl 2):S1?S45.

PDF Image

Value
Not applicable

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LABCORP

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